

Delirium



Section I: Scenario Demographics

Scenario Title:	Delirium
Date of Development:	(21/04/2018)
Target Learning Group:	☐ Juniors (PGY 1 - 2) ☐ Seniors (PGY $\geq$ 3) ☒ All Groups

Section II: Scenario Developers

Scenario Developer(s):	Professor Victoria Brazil, Dr Nemat Alsaba, Dr Rebecca Shaw
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Section III: Curriculum Integration

Learning Goals & Objectives	
Educational Goal:	To practice the focused assessment and management of an elderly patient with delirium.
CRM Objectives:	1. Obtain early control of situation to ensure safety of patient, staff and department. 2. Coordinate staff involved in the management of a delirious patient.
Medical Objectives:	1. Consider ways to facilitate patient assessment when patient is agitated and uncooperative. 2. Seek possible causes of delirium during the assessment. 3. Demonstrate practical strategies (pharmacological and non-pharmacological) for behavioral management of a delirious geriatric patient in ED.

Case Summary: Brief Summary of Case Progression and Major Events

An 81-year-old (wo)man is brought to the ED by her/his friend as she/he is confused and agitated. In the ED, her/his confusion worsens. Initially she/he is fidgety but as the case progresses she/he becomes more agitated and confused. She/he will be fairly uncooperative, moving around and not able to follow many commands. The participants should be looking for a source of infection and evidence of any recent trauma. They are expected to use both non-pharmacological and safe pharmacological options in order to control the situation, ensure patient safety, and facilitate investigations.

References

<https://gempodcast.com/2015/11/11/diagnosing-and-managing-delirium-in-older-adults/>

<https://www.safetyandquality.gov.au/wp-content/uploads/2014/11/A-better-way-to-care-Actions-for-clinicians.pdf>

Tropea, J., Logiudice, D., & Kelly, L. (2017). People with dementia in the emergency department: Behavioral symptoms and use of restraint. *Emergency Medicine Australasia*, 29(5), 605-606





Section IV: Scenario Script

A. Clinical Vignette: To Read Aloud at Beginning of Case			
Patient is sitting on the edge of the ED bed, looking perplexed. She/he is fidgeting and not concentrating on the questions being asked, she/he is staring around the room, looking in his/her bag and picking at the BP cuff and bed sheet. The ED nurse is attempting to do some baseline vital signs on the patient. Participants asked by ED RN "Could you please go and assess Mr. (s) Thompson? She/he has just been brought in to the ED by ambulance after a friend found her/him confused at home".			
B. Scenario Cast & Realism			
Patient:	☐ Computerized Mannequin ☐ Mannequin ☒ Standardized Patient ☐ Hybrid ☐ Task Trainer	Realism: <i>Select most important dimension(s)</i>	☒ Conceptual ☐ Physical ☒ Emotional/Experiential ☐ Other: ☐ N/A
Confederates	Brief Description of Role		
Patient (SP)	You are an elderly (wo)man who is usually independent and with reasonable cognitive function. You have been unwell for 36 hours – not feeling like eating and drinking and not wanting to go out like you normally would. Right now you are feeling a little unwell but do not have any pain or any specific symptoms to complain about. You are also feeling frightened and perplexed because you don't know where you are, you don't remember how you got here, you don't know who the people are in this place, and you just want to go home and have a glass of "sherry ". However, you cannot recall where you live. You are a bit cross about being asked so many questions and you are having trouble concentrating on them and what the answers might be. You are feeling a bit upset that you don't know all the answers. In fact after a while, the only answers you know are your name and your birthday, but you can't remember which year you were born. You don't answer any more questions after a few minutes. You are most concerned to find your glasses and your mother – you are sure you are lost and you'd like to find her so she can take you home. As the minutes pass, you become more agitated and uncooperative and openly hostile to the participants who wish to examine you, turning away, trying to get off bed and pushing their hands away. You repeat "Don't do that!" a number of times while they are talking to you. The nurse is able to reassure/distract you to a certain extent. It's likely some sedative medication will be provided. You are very suspicious/ paranoid that this is poison and won't drink it. It's likely then they will give you medication in the drip		
ED RN	Provides initial handover. To assist at the bedside.		

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Friend	Participants should be aiming to find out about baseline level of functioning, recent illnesses, current medicines/changes to Rx and any concern re alcohol misuse. When asked, inform them of the following details regarding the patient: PMHx – HTN, osteoporosis, recurrent UTIs, depression Medications – acetaminophen, amlodipine, citalopram Nil allergies SH – lives alone, independent, drinks 3 glasses of sherry a day She/he is usually well and manages well on her/his own		
C. Required Monitors			
☒ EKG Leads/Wires	☐ Temperature Probe	☐ Central Venous Line	
☒ NIBP Cuff	☐ Defibrillator Pads	☐ Capnography	
☒ Pulse Oximeter	☐ Arterial Line	☐ Other:	
D. Required Equipment			
☒ Gloves	☒ Nasal Prongs	☐ Scalpel	
☒ Stethoscope	☒ Venturi Mask	☐ Tube Thoracostomy Kit	
☐ Defibrillator	☒ Non-Rebreather Mask	☐ Cricothyroidotomy Kit	
☒ IV Bags/Lines	☒ Bag Valve Mask	☐ Thoracotomy Kit	
☒ IV Push Medications	☐ Laryngoscope	☐ Central Line Kit	
☐ PO Tabs	☐ Video Assisted Laryngoscope	☐ Arterial Line Kit	
☐ Blood Products	☐ ET Tubes	☐ Other:	
☐ Intraosseous Set-up	☐ LMA	☐ Other:	
E. Moulage			
Patient sitting on the edge of the bed in their pyjamas. Graze/abrasion to left forehead.			
F. Approximate Timing			
Set-Up:	10 min	Scenario:	20 min
		Debriefing:	20 min



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Section V: Patient Data and Baseline State

A. Patient Profile and History			
Patient Name: Sandra / Sam Thompson		Age: 81	Weight: 55kg
Gender: ☐ M ☐ F ☒ Either		Code Status: Presumed full	
Chief Complaint: Confusion			
History of Presenting Illness: Patient brought in by ambulance from home – friend went to their house after they didn't meet her at the shops as previously arranged. Found patient confused, still in their pyjamas, having been incontinent of urine. S/he has a decent abrasion/ graze on his/her head			
Past Medical History:	Hypertension	Medications:	Acetaminophen
	Osteoporosis		Citalopram
	Recurrent UTIs		Amlodipine
	Depression		
Allergies: Nil Known			
Social History: Widow, lives alone, independent normally. Non- smoker. Drinks 3 glasses of sherry a day.			
Family History: sister has IHD			
Review of Systems:	CNS:	Confused and agitated. Moving all 4 limbs.	
	HEENT:	Unable to assess	
	CVS:	Unable to assess	
	RESP:	Unable to assess	
	GI:	Unable to assess	
	GU:	Unable to assess	
	MSK:	Unable to assess	INT:
B. Baseline Simulator State and Physical Exam			
☒ No Monitor Display ☐ Monitor On, no data displayed ☐ Monitor on Standard Display			
HR: unable	BP: unable	RR: unable	O ₂ SAT: unable
Rhythm: unable	T: unable	Glucose: unable	GCS: 14 (E4 V4 M6)
General Status: Confused and mildly agitated			
CNS:	Confused and mildly agitated. Moving all 4 limbs		
HEENT:	Graze/abrasion to left forehead		
CVS:	Unable to assess		
RESP:	Unable to assess		
ABDO:	Unable to assess		
GU:	Unable to assess		
MSK:	Unable to assess	SKIN:	Unable to assess



Section VI: Scenario Progression

Scenario States, Modifiers and Triggers			
Patient State	Patient Status	Learner Actions, Modifiers & Triggers to Move to Next State	
1. Baseline State Patient initially refusing vital signs to be taken but will allow if appropriately approached. Rhythm: sinus rhythm HR: 86/min BP: 150/70 RR: 14/min O ₂ SAT: 95 % T: 37.3 °C	Patient confused and slightly agitated.	Learner Actions - ☐ Patient reassurance - ☐ Check vitals - ☐ Cap sugar (6.4mmol/L) - ☐ Attempt history targeted to presenting complaint - ☐ Request family member/collateral history	Modifiers <i>Changes to patient condition based on learner action</i> - Patient allows vitals to be taken when reassurance provided - Confederate nurse to offer reassurance and take vitals if not done by sim participants Triggers <i>For progression to next state</i> - All actions completed or after 5 mins → 2. Worsening Confusion
2. Worsening Confusion No change in vital signs.	Patient answers a couple of questions then stops interacting. Seems distracted.	Learner Actions - ☐ Collateral history - ☐ Verbal de-escalation - ☐ Environmental management - ☐ Offer oral sedative - ☐ IV access with broad panel of blood work drawn - ☐ IV/IM sedative	Modifiers - Patient allows IV access when distracted - Confederate nurse to prompt for IV access if not volunteered Triggers - Outstanding de-escalation by team or sedative given → 3. De-escalation
3. De-escalation No change in vital signs	Patient asking about her mother and wants to go home, but more settled and cooperative.	Learner Actions - ☐ Make plans for further investigations – ECG/CXR/urine/CT head - ☐ Make plans for admission - ☐ Possible IV fluid.	Modifiers - Confederate nurse to prompt by asking about patients disposition END CASE PRN

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Section VII: Supporting Documents, Laboratory Results, & Multimedia

Laboratory Results

No blood work back during case.

Images (ECGs, CXRs, etc.)

None given during case.



Section VIII: Debriefing Guide

General Debriefing Plan	
☐ Individual	☒ Group
☐ With Video	☒ Without Video
Objectives	
Educational Goal:	To practice the focused assessment and management of an elderly patient with delirium.
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Sample Questions for Debriefing	
1. How did you approach this uncooperative patient? 2. What did you think might be going on with the patient? 3. What extra information did you need to enable you to sort this patient out? 4. Delirium has wide range of causes. We are interested to hear how you approached it in this case. 5. How can you safely contain this situation to ensure patient and staff safety? 6. What would have been the next step in diagnosing and managing this patient in the emergency department? 7. How would you transfer knowledge from today's simulation into your workplace?	
Key Moments	
Assessment of confused and agitated geriatric patient	
Attempted non-pharmacological options to contain confused and agitated geriatric patient	
Choice of sedative agent for delirious elderly patient	